

إِسْمُ الْمَرِيضِ رُبَاعِيًّا : مدحت صالح منصور سليمان

العمر(تاريخ الميلاد) : 21-02-1995 المهنة : حاصل على دبلوم

الإحصائي الاجتماعي
التوقيع: (_____)

التوقيع: (-----)

التعليق على التشخيص :-----

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

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ملاحظات :

المجلس الطبي المتخصص

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Name: مدحت صالح منصور سليمان National ID: 29502213200032	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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A patient of prostate cancer for Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





رئيس الوحدة
الدكتور
أ. د. هدى إسماعيل - أ. د. بشار طه
د. حبيبة النوري - د. م. عادل - د. مصطفى أحمد
د. محمد الشافعي - د. أحمد علي شافعي

بالتأكيد..

معمل
البرج

Pathology Unit

فرع : B-El Haramein Medical Center

26496 السن : 32

اسم المريض : مدحت صالح منصور

06-07-2026 تاريخ الحضور :

محول من أ. د. محمد محمود حسن

15-07-2026 تاريخ الاستلام :

رقم الزيرة : 15126505743

NATURE OF SPECIMEN:

Lt. Radical orchidectomy

GROSS:

Lt. Radical orchidectomy specimen 9x5x2.5 cm with intact covering tunica and attached spermatic cord 4.5 cm, long. Serial sections revealed a firm whitish mass 4.5x3.7cm having greyish white cut section showing areas of hemorrhage and breaking down.

MICROSCOPY:

Sections showing mixed germ cell tumor with dominant (about 90%) of tumor formed of compartments of polygonal cells with ill-defined cells borders and vesicular nuclei. No detected syncytiotrophoblastic cells. Compartments are separated by incomplete fibrous septa infiltrated by small lymphocytes. In areas, embryonal carcinoma component (about 10%, slide D) where tumor is formed of malignant tumor formed of sheets and nodules of atypical epithelioid cells and gland like structures separated by thin fibrous bands. Tumor cells have large vesicular nuclei with prominent nuclei and pale eosinophilic cytoplasm with ill-defined cell borders. There is moderate to marked anaplasia with active mitosis and related areas of necrosis. No evidence of lymphovascular or perineural invasion. Nearby Seminiferous tubules show focal intratubular germ cell neoplasia (IGCN). Tumor reaches to tunica but without penetrating it. Spermatic cord margin is negative for tumor infiltration. Confirmation of the mixed components of this germ cell tumor by marker study (PLAP and CD30) may be advised.

DIAGNOSIS:

Testicular mass, radical orchidectomy: Mixed germ cell tumor (dominantly classic seminoma with minor components of embryonal carcinoma). Margins of excision are free of tumor infiltration.

Neveen Tahoun

Akram Nauh

Prof. Dr. Neveen S. Tahoun

Prof. Dr. M. Akram Nauh

Authentication Date: 11-07-2026

رجاء الاحتفاظ بالترشيح البرقية كسجل دائم للحقة - لا تقوم بالعمل بتعليم ترشحات الشجع الا للحالات السرطانية فقط ويتم الاحتفاظ بهذه التراكبات لمدة عام بعد تشخيص الحالة

Patient Name: Medhat Saleh Mansour

Date: 27 March 2026

US & DUPLEX DOPPLER OF THE SCROTUM

RIGHT HEMISCROTUM:

- Normal site, size (measuring about: 3.5 x 2.8 x 1.8 cm) with normal shape, echopattern and vasculature of the testis, no focal lesions. Thin normal tunica and intact mediastinum testis.
- Normal size shape, echopattern and vasculature of the epididymis, no focal lesions.
- The panpinform plexus of veins is within average size.
- Minimal clear hydeocele is noted.

LEFT HEMISCROTUM:

- Relatively enlarged in size (measuring about 4.3 x 3.3 x 5.3 cm), showing heteroeognous echopattern , a rather defined heterogenous solid mass lesion is seen with cystic degeneration, measuring about 3.2 x 2.2 cm showing internal vascularity on color Doppler.
- Moderate dilatation of the panpinform plexus of veins is noted (+/- 3.7 mm) reaching lower pole of the testicle, show signiciant reflux on straining in color flow maping and in Doppler spectral wave forms even in supine position.

OPINION:

- Left testicular mass lesion for further assessment.
- Moderate left sided refluxing varicocele.

Sincerely
Dr. Mahmoud M. Abd Allah

Pat. Name: Medhat Saleh Mansour Soliman	Pat.ID: 789947
Date: 20/07/2026	Pat. Age: 31 Y(s)
PROF. DR. MOHAMED MAHMOUD HASAN	

MSCT Chest, Abdomen, and Pelvis with Contrast

Findings:

• Chest:

- Lungs are clear with **no pulmonary nodules, masses, or infiltrates.**
- No pleural effusion or thickening.
- Mediastinum is unremarkable with **no lymphadenopathy.**
- Heart and great vessels are normal in size and configuration.
- Osseous structures are intact with **no lytic or sclerotic lesions.**

• Abdomen:

- The liver is of average size with homogenous texture. **No focal lesions** or contour irregularities. Patent portal vein and is main branches as well as its main tributaries. No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size , and shape of the spleen shows homog. parenchyma .
- Normal size , and shape of the pancreas , No peripancreatic collection. No mass lesions .
- Normal appearance of both kidneys, as regard size, site, and shape shows normal parench. thickness . No stones or dilated P.C.S...
- No intra-abdominal lymphadenopathy or free fluid.

• Pelvis:

- Prostate is unremarkable.
- Urinary bladder is normal.
- No pelvic lymphadenopathy or masses.
- Visualized bones of the pelvis show **no evidence of metastatic disease.**

Impression:

- History of orchidectomy with no evidence of metastatic disease in the neck, chest, abdomen, or pelvis.

Sincerely,
DR. TAREK M. MANSOUR. M.D.

Tarek M. Mansour